
Discharge Summary

Patient: Friedrich Kremer

Date of Birth: 10/23/1957

Admission Date: 10/14/2024

Discharge Date: 10/21/2024

Attending Physician: Dr. Annette Hoffmann, MD

Dear Dr. Meier,

We are writing to inform you in detail about the inpatient stay of Mr. Friedrich Kremer, DOB 10/23/1957, admitted October 14 to October 21, 2024.

Mr. Kremer was admitted for optimization of poorly controlled type 2 diabetes with newly identified peripheral arterial disease. Current regimen at admission: metformin 1000 mg twice daily and gliclazide MR 120 mg. Self-monitored glucose values had been consistently 250–320 mg/dL for 3 months. HbA1c on admission 9.8%.

Primary Diagnosis: Type 2 diabetes mellitus with peripheral angiopathy (E11.5)

Past Medical History: Type 2 diabetes diagnosed 2011. Hypertension on perindopril 8 mg and indapamide 1.5 mg. Dyslipidaemia on rosuvastatin 10 mg. No prior cardiac events or revascularisation. Active smoker, 35 pack-years — intensive cessation counselling provided. Alcohol approximately 10 units per week — brief intervention given by ward pharmacist. Self-employed plumber, lives with wife.

Physical Examination on Admission: Height 178 cm, weight 96 kg, BMI 30.3. BP 156/92 mmHg, HR 82 bpm, temperature 36.9°C. Femoral pulses palpable bilaterally, popliteal and pedal pulses markedly diminished bilaterally. Feet cool to palpation, no ulceration or discolouration. Monofilament sensation reduced in both feet to the ankle. Vibration sense absent at first metatarsal head bilaterally. ABI performed day 2: right 0.72, left 0.78.

Investigations: Duplex ultrasound lower extremity arteries: bilateral femoral-popliteal atherosclerotic disease without critical stenosis at rest. Fundoscopy: mild non-proliferative diabetic retinopathy bilaterally. UACR 68 mg/g — microalbuminuric range. Renal function within normal limits.

Hospital Course: Gliclazide discontinued; insulin glargine 14 units at bedtime initiated with 2-unit titration every 3 days if fasting glucose above 130 mg/dL. Metformin

continued. Aspirin 100 mg added. Rosuvastatin increased to 20 mg. NRT 21 mg/24h patches prescribed.

Diabetic Foot and Vascular Education: Comprehensive foot education delivered by the diabetes specialist nurse on day 4 — daily inspection, appropriate footwear, nail care, early warning signs. Written foot care leaflet provided. Podiatry referral arranged for baseline assessment.

Lifestyle Modification: Smoking cessation was addressed as the highest-priority intervention given the synergistic effect on peripheral vascular disease progression. FRAMES-based intervention delivered by ward pharmacist. The patient expressed genuine motivation to quit, citing his vascular health as the primary motivator. He was enrolled in the hospital cessation programme with a first appointment within 1 week of discharge. Alcohol intake was addressed — the patient agreed to reduce to a maximum of 14 units per week as an initial target.

Medication at Discharge: Insulin glargine 14 units at bedtime, metformin 1000 mg twice daily, perindopril 8 mg, indapamide 1.5 mg, rosuvastatin 20 mg (increased), aspirin 100 mg, NRT 21 mg/24h patches.

Follow-up: GP in 2 weeks for blood pressure and fasting glucose. Diabetology in 4 weeks for insulin titration. Vascular surgery referral initiated. Smoking cessation first session within 1 week. Mr. Kremer was given detailed written instructions on insulin titration, hypoglycaemia management, and foot care.

Structured Smoking Cessation Intervention:

Smoking cessation was identified as the single most impactful modifiable intervention for Mr. Kremer given his 35 pack-year history and the confirmed peripheral arterial disease, where continued smoking markedly increases the risk of limb loss and cardiovascular events. A FRAMES-based brief intervention was delivered by the ward pharmacist on day 3. Mr. Kremer expressed his most genuine motivation to quit in years, citing concern about amputation risk and the desire to remain physically active in his trade. Nicotine replacement therapy with 21 mg/24-hour transdermal patches was prescribed at discharge. He was enrolled in the hospital's structured smoking cessation programme with a first appointment booked within 1 week of discharge.

Peripheral Arterial Disease Education and Vascular Referral:

Mr. Kremer received a comprehensive education session on peripheral arterial disease from the vascular diabetes specialist nurse on day 5. Topics covered: pathophysiology of PAD in diabetes; the significance of his ABI values and implications for limb risk; daily foot inspection techniques, appropriate footwear, nail and skin care; early warning signs of critical ischaemia requiring emergency care; and the importance of attending the vascular surgery follow-up. A vascular surgery referral was submitted and an appointment confirmed within 6 weeks for further assessment and discussion of revascularisation options if clinically indicated.

Alcohol intake of 10 units per week was also addressed with a structured brief intervention. Mr. Kremer acknowledged that his alcohol intake contributed to caloric excess and suboptimal metabolic control and agreed to reduce to a maximum of 14 units per week as an initial target, with a view to further reduction.

Dr. Annette Hoffmann, MD

Senior Physician, Internal Medicine